

DETAILED PRACTICAL GUIDE

Innovative approaches to GBV service provision in emergencies

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Important: This is a detailed unofficial reading guide, not an official translation. Consult the original publication for complete requirements and context.

Based on the original publication

<https://www.unicef.org/media/83381/file/Innovative-GBV-Service-Provision-Emergencies%20.pdf>

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01 Purpose and scope

- Examples and safeguards for adapting gender-based-violence services during emergencies, lockdowns and movement restrictions.

02 Key guidance

- Remote case management should be offered only after checking device access, surveillance risks, privacy, connectivity and the survivor's preferred contact method.
- Use neutral appointment names, agreed code words, safe times and rapid-exit plans; never assume a personal phone is private.
- Maintain in-person options for people without safe technology, literacy, disability access, money, identification or stable connectivity.
- Pharmacies, health facilities, cash programmes and community networks may support discreet referrals only with training and strict confidentiality.
- Digital tools should collect the minimum data, use role-based access, avoid unsafe cloud backups and have a breach-response plan.
- Staff need protocols for interrupted calls, imminent danger, child safeguarding, cross-border referrals and technology failure.
- Monitor whether adaptations exclude groups or increase control by perpetrators; convenience for providers is not evidence of safety.
- Document innovations, survivor feedback and adverse effects so temporary measures improve rather than weaken service quality.

03 How to use this guide

- This guide turns the source publication into a practical reading aid. It is designed for survivors, trusted supporters, health and social-service staff, and programme managers. It does not replace emergency services, medical care, legal advice, or the complete source publication.
- Use only the parts that fit the person's situation. Preserve her choice, privacy and pace. If a suggested action could expose browsing, calls, documents or location to an abusive person, first choose a safer device, place or time.

04 Survivor-centred principles

- Safety comes first. Ask what would make the next hours or days safer and avoid steps that may provoke retaliation. Never require a disclosure in order to offer basic help.
- Respect autonomy and informed consent. Explain realistic options, benefits, limits and possible risks; the survivor decides what happens next unless an immediate legal safeguarding duty applies.
- Protect confidentiality. Collect the minimum information, restrict access, agree on safe contact methods and explain any limits before information is shared.
- Provide equitable access. Consider disability, language, age, migration status, poverty, rural isolation, sexual orientation and other barriers without stereotyping.

05 Practical response sequence

- Begin with immediate safety and urgent health needs. Ask privately, without the suspected perpetrator present, whether it is safe to talk and whether the person needs emergency care.
- Listen without interrogation. Validate what happened, avoid blame, and do not demand a chronological account or proof. Record only what is necessary and use the survivor's own words.
- Offer a small number of verified options: medical care, safe accommodation, psychosocial support, legal information, police or protection services where appropriate. Explain that accepting one service does not require accepting all others.
- Make a warm referral when consent is given: confirm the service is open, accessible and safe; explain what to expect; and avoid sending the survivor repeatedly between agencies.
- Agree on follow-up and a safe way to communicate. Do not leave identifying messages, send revealing notifications or store files under obvious names if devices may be monitored.

06 Safety and quality checks

- Do not confront the suspected perpetrator, mediate a violent relationship, promise an outcome you cannot control, or pressure a survivor to leave immediately.
- Do not use unverified phone numbers or referrals. Services may close, change eligibility rules or share information in unsafe ways; confirm details before publication or referral.
- Organisations should train staff, supervise difficult cases, create confidential documentation rules, maintain updated referral maps and offer staff support to reduce burnout and harmful practice.

Action checklist

- Is it safe to speak, read or keep this document right now?
- Have urgent injuries, pregnancy concerns, sexual-violence care and suicide risk been considered?
- Has the survivor chosen what information may be recorded or shared?
- Are interpretation, disability access, childcare, transport and cost barriers addressed?
- Are referrals verified and is there a safe follow-up plan?
- Has the person been told that the violence is not her fault and that she can decline any option?